Sorafenib: differentiated thyroid cancer

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Continuous oral treatment

Active medicines: Sorafenib

Emetogenic risk: Minimal to low (oral)

Medicine	Dose / route	When
Antiemetic plan for current oral dose	Individual prescription	As agreed with oncology/pharmacy Review QT prolongation and all interactions; no automatic intravenous (IV) chemotherapy script.

Additional phase-specific support

- Take with water, without food or with a low- or moderate-fat meal. For a high-fat meal, take sorafenib at least 1 hour before or 2 hours afterwards. Follow the prescribed morning and evening doses; never change the dose yourself.
- Consider blood count, kidney and liver tests at Day 14 of the first month. Repeat these bloods and thyroid tests every 28 days. Check blood pressure weekly in month 1, then every 28 days; check international normalised ratio (blood-clotting test) (INR) as indicated if taking warfarin.
- Tell the team before surgery or dental procedures. Report painful palms or soles early; skin care and early dose review can help prevent severe hand-foot reactions.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00294 v4 · pTreatment / supportive care · NCCP regimen and NCIS antiemetic guidance

Sources checked 2026-11-14 · SACTCheck v0.84.50 · Regimen-level drafts; prescriber and oncology-pharmacy verification required